جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : ساميه حمزاوي عبدالهادي سليمان
المحافظة : الجيزه **الحي / المدينة :** العياط **العنوان :** بهبيت
رقم البطاقة : 27702062100601 **رقم التليفون :** 01152005760 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 06-02-1977 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساميه حمزاوي عبدالهادي سليمان
National ID: 27702062100601

GUSTAVE ROUSSY INTERNATIONAL EGYPT

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

MDT

Name : [redacted] Sex :

Patient Age : 42 Y

Phone [redacted]

Family History : -ve

Marital status : [redacted]

NSS Done : [redacted]

Menopausal status : [redacted]

BMI : [redacted]

GROUP : [redacted]

Initial STAGE : [redacted]

REASON OF MDT : [redacted]

Date of MDT : 27/07/2024

Clinical data

Breast Imaging :
[redacted] of defined mass about 5* 4 cm felt as fibro fatty tissue. Core B. Ix, pathological target 2.4*1.4 cm

Mets Workup :
MILLIMETREC PULM NODULES
TRIPHASIC CT NO MTS
BONE SCAN FREE

ER	PR	Her 2 Neu	SISH	KIG7
7/8	8/8	0		13

T	N	M
3	1	0

Sonomammography

PATIENT HISTORY
pt received NACT AC x4 +taxotere x4 ended 13/5/2024
o/e: lt breast & axilla free (no palpable mass)
SIGNIFICANT REGRESSIVE SIZE OF MASS NOW 3CM COMPARED 9CM
BCS+SLNB : free margin
LN 0/4

MDT OUTCOME & PLAN

FOR ADJUVANT HT (ACC TO E2 /FSH)
PORT

SIGNATURE

MEDICAL ONCOLOGY	[redacted]	RADIOLOGY	[redacted]
SURGERY	[redacted]	PATHOLOGY	[redacted]
Psychiatric	[redacted]	Nutrition	[redacted]

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Name: سامية حمزاوي عبد الهادي سليمان **MR. :** 5646	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

The patient is a 49-year-old individual with a diagnosis of left breast invasive ductal carcinoma, estrogen receptor 7/8 positive, progesterone receptor 8/8 positive, HER2-negative, clinical stage T3 N1 M0. Neoadjuvant chemotherapy was administered consisting of 4 cycles of anthracycline (AC) and 4 cycles of taxane (TXT), completed on 15 March 2024. The patient subsequently underwent left breast-conserving surgery with sentinel lymph node biopsy on 8 June 2024, revealing 0 positive nodes out of 4 examined. Post-operative management included placement of a port and initiation of tamoxifen 20 mg daily beginning in July 2024. The patient is also taking calcium 1 tablet daily (one Alpha-CT capsule) and has scheduled imaging studies: free-standing CT in August 2025, free-standing MRI of the brain in December 2025, and free-standing CT of the chest/abdomen/pelvis (CXR PAUS TVUS) in August 2025. The current plan is continuation of tamoxifen therapy and adherence to the upcoming imaging schedule.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : ايه مدحت بدوي محمد

يعتمد ,

Patient Name	: الأستاذه / رشا رمضان سعد شعيل	Request No.	: 13006840
Patient No.	: 28704191401606	Request Date	: 21/10/2023
Path No.	: 3623/23	Verification Date	: 01/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - مباشره
Age	: 36 Y	Department	: Out Patient
Refereing Doc	:	Registered by	: Ali mostafa

Pathology Report

Clinical Data: Left breast swelling.
Nature of Specimen: Tru-cut biopsy, collected at 21/10/2023, 1:30 pm.

Gross Examination:
- Multiple grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive pleomorphic lobular carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
* Ductal carcinoma in situ: No.
* Perineural invasion: No.
* Angioinvasion: No.
* IPT (Positive control showed appropriate reactivity):
- E.Cadherin: Negative.
- ER: Strong nuclear reaction in 80% of tumor cells (Allred score 8/8).
- PR: No nuclear reaction in the tumor cells (Allred score 0/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 20% nuclear reaction of tumor cells.

Diagnosis:
Left breast swelling, Tru-cut biopsy:
Invasive pleomorphic lobular carcinoma, Grade II .
IPT:
-E.cadherin: Negative.
- ER: Positive (score 8/8).

Verified by :
DR.Menna Nabil

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - أوريون تكيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / رشا رمضان سعد شعبان	Request No.	: 13006840
Patient No.	: 28704191401606	Request Date	: 21/10/2023
Path No.	: 3623/23	Verification Date	: 01/11/2023
Sex	: Female	Financial Category	: اورام ثدى السلام - مباشره
Age	: 36 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Ali mostafa

Pathology Report

- PR: Negative (score 0/8).
- Her2: Positive (score 3+).
- Ki67: Positive In 20% of tumor cells.

Signature
prof.Dr Gamal Abdelatty
Dr. Israa Magdy.

Verified by :
DR.Menna Nabil

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Coralche - Misr El Qadema - Cairo, Egypt

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	:	Request No.	: 13006190
Patient No.	: 27702062100601	Request Date	: 12/09/2023
Tafa No	: 3076/23	Verification Date	: 25/09/2023
Sex	: Female	Financial Category	:
Age	: 48 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed youss

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast mass.

Nature of Specimen:Tru-cut biopsy , collected at 12/9/2023 , 1:30 pm.
Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
IPT (Positive control showed appropriate reactivity):
ER: Moderte nuclear reaction in 70% of tumor cells (Allred score 7/8).
PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8).
Her2: No membrane reaction in the tumor cells (score 0).
Ki67: 19 % nuclear reaction of tumor cells.

Conclusion:
Left breast mass, Tru-cut biopsy:
Invasive duct carcinoma, GII.
IPT:
ER: Positive (score 7/8).
PR: Positive (score 8/8).
Her2 Negative (score 0).
Ki67: Positive in 19% of tumor cells.

Signature
Prof.Dr Lobna Omar.

Verified by :

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	: الأستاذه / ساميه حمزاوى عبدالهادى سليمان	Request No.	: 13004512
Patient No.	: 27702062100601	Request Date	: 08/06/2024
Path No.	: 1954/24	Verification Date	: 07/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 47 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Left breast cancer, post NACT.

Nature of the specimen:
Left wide local excision and sentinel lymph nodes biopsy.

Gross Examination:
Two specimens were recieved as follow:
- Left wide local excision specimen measured 4x4x3 cm, with no covering skin.
• Tumor extent: A wire guided greyish white tumor bed site.
o Size: 3.3x2x1.7 cm.
• Distance from margins: 0.2 cm from the least lateral & medial margins.
- Sentinel lymph nodes: Fibrofatty tissue peice measured 4x4 cm, entangling 4 nodules, up to 1x1x0.5 cm.

• Frozen results:
- Free sentinel lymph nodes, S.L.N (0/4).
- Free surgical margins.

• 10 Blocks:
1- Superior margin., 2- Inferior margin., 3- Lateral margin., 4- Medial margin., 5- Deep margin., 6:9- T.B.S., 10- SLNs., 10.
- Gross was done by; Dr. Mohamed Turkey.

Microscopic examination:
• Histological type: Residual viable invasive duct carcinoma.
• Grade (differentiation+ nuclear pleomorphism + mitosis): G II(3+2+1).
- Actual tumor size: 1.8x1 cm .

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

H, S Patient ID: 123024781 DOB: 06-Feb-1977 Date: 26-Nov-2023
Patient Lastname: H
Patient Firstname: S
HIS Patient ID: 123024781
RIS Patient ID: 342712
Date of Birth: 06/02/1977

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

REPORT TEXT

Nuclear Medicine Unit ???? ???? ?????

99mTc-MDP BONE SCAN

Brief history: 45 years old female patient diagnosed with managed breast cancer.
Desired Information: - skeletal survey.
Technique:
Delayed Whole body imaging in anterior and posterior projection performed 2 hours after I.V administration of 20 mCi 99m Tc-MDP in the following manner:

Report:

- No hot spot that account for osseous deposits could be detected allover the surveyed skeleton.
- Spondylo-degenerative changes are seen over dorso-lumbar vertebrae.
- Pronounced osteoarthrosis noted over both knee joints.
- Both kidneys are normally visualized.

Conclusion:

Negative study for the detection of any definite metastatic osseous deposits.

Thanks for reference

Nuclear Medicine Unit
Aya Ashraf, MSc.
Mahmoud Akram
Rawan Sherra

REPORT TEXT ENDE

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Total DLP (mGy.cm):

Patient Name: Samia Hamzawy Abd Al Hady	Date : 13/11/2023
Exam : TRIPHASIC ABDOMEN &PELVIS	Age : 46 Year(s)
Medical ID: 3744/2023	Pat. ID: 42649

POST CONTRAST MULTI-SLICE CT SCAN OF THE ABDOMEN & PELVIS REVEALED WITH TRIPAHSIC LIVER: -

Compared to 03/10/2023

Findings:

- The liver is average in size. The previously mentioned left hepatic lobe segment IVa subtle hypodense focal lesion is inconspicuous in today's study.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with multiple mural calcified plaques noted.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Anterior abdominal wall fat containing hernia.
- Lower chest cuts shows few stationary few right lung subcentimetric nodules largest at middle lobe measuring 4 mm.

The scanned skeleton shows stationary:

- Few L4 and L5 tiny subcentimetric lytic foci are noted.
- Right femoral neck sclerotic lesion mostly representing bone island.
- No vertebral structural collapse or extra-osseous soft tissue component.

CT Scan
[[DOCTYPE:CT_SCAN]]

Mlnistry Of Health
Specialized Medical Centres
Dar El Salam Cancer Hospital
Radiology Department

Total DLP (mGy.cm):

Patient Name: Samia Hamzawy Abd Al Hady	Date : 13/11/2023
Exam : TRIPHASIC ABDOMEN &PELVIS	Age : 46 Year(s)
Medical ID: 3744/2023	Pat. ID: 42549

OPINION:

- *The previously mentioned left hepatic lobe segment IVa subtle hypodense focal lesion is inconspicuous in today's study.*
- *Stationary rest of the study.*

Dr. Ali Saber, MSc.

Signature Prof. Mohamed Gomaa

29/11/2023 08:41 ص

Dr. M. Gomaa, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Samia Hamzawy Abd Al Hady	Date : 11/03/2024
Exam : MAMMOGRAPHY BOTH	Age : 47 Year(s)
Medical ID: 3744/2023	Pat. ID: 42549

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast malignancy under neoadjuvant chemotherapy.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ spiculated irregular mass lesion, of high density measuring 89 X 72 mm in CC dimension.
- Right breast UOQ well defined medium density lesion with internal foci of macro-calcifications.
- Complementary ultrasound revealed regressive course of the left breast irregular spiculated hypoechoic mass lesion, , seen 11 o'clock position, measuring 14 X 15 X 32 mm.
- Non appreciated forenoted left breast 12 o'clock lesion.
- Stable right breast 11 o'clock well defined oval shaped hypoechoic lesion measuring 10 X 6 mm.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific axillary LNs.

Opinion:

- Regressive course of the left breast malignant looking mass lesion denoting partial therapeutic radiological response (BIRADS 6).
- Stable right breast benign looking lesion likely involuting fibroadenoma (BIRADS 3).

Yours truly

Signature Dr. Mohamed Mohsen

11/03/2024 12:38 PM

Signature Dr. Mohamed Mohsen

11/03/2024 12:46 PM